

1. Administrative & Study Identification

Full Study Title A Phase III Study Evaluating Glofitamab in Combination With Gemcitabine + Oxaliplatin vs Rituximab in Combination With Gemcitabine + Oxaliplatin in Participants With Relapsed/Refractory Diffuse Large B-Cell Lymphoma **Official Regulatory Title** A Phase III, Open-Label, Multicenter, Randomized Study Evaluating the Efficacy and Safety of Glofitamab in Combination With Gemcitabine Plus Oxaliplatin Versus Rituximab in Combination With Gemcitabine and Oxaliplatin in Patients With Relapsed/Refractory Diffuse Large B-Cell Lymphoma **Short Title/ Acronym** STARGLO Study **Protocol Number** G041944 **NCT Number** NCT04408638 **Posting ID** 387901819878840696 **Sponsor/Company Name** Roche (Hoffmann-La Roche) **Investigational Product** Glofitamab (Columvi) + GemOx **Phase of Development** PHASE3 **Trial Status** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Medical Monitor** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of glofitamab in combination with gemcitabine plus oxaliplatin (Glofit-GemOx) compared with rituximab plus GemOx (R-GemOx) by assessing Overall Survival (OS). **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Complete Response (CR) rate, Objective Response Rate (ORR), Duration of Response (DOR), and safety profiles including Cytokine Release Syndrome (CRS) incidence.

2.2 Background & Rationale To address the unmet medical need in patients with relapsed or refractory diffuse large B-cell lymphoma (R/R DLBCL) who have failed at least one prior line of therapy and are ineligible for autologous stem cell transplantation. Glofitamab, a CD20xCD3 bispecific antibody, provides an off-the-shelf, fixed-duration therapeutic alternative to cellular products like CAR T-cell therapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Rituximab + Gemcitabine + Oxaliplatin) **Blinding:** Open-label **Randomization:** Randomized, 2:1 (Stratified by 1 vs ≥2 previous lines of therapy and relapsed vs refractory status)

3.2 Planned Enrollment & Duration Target **\$N\$:** 270 patients **Number of Sites:** 62 centers in 13 countries across Asia, Australia, Europe, and North America **Estimated Study Start:** 23-Feb-2021 **Estimated Primary Completion:** 31-Mar-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically confirmed diffuse large B-cell lymphoma (DLBCL), not otherwise specified. 3. Relapsed/refractory (R/R) disease defined as: Relapsed = disease that has recurred ≥ 6 months after completion of the last line of therapy; Refractory = disease that either progressed during the last line of therapy or progressed within 6 months (<6 months) of the last line of prior therapy. 4. At least one (≥ 1) prior line of systemic therapy. (*Note: Participants who have failed only one prior line must not be a candidate for high-dose chemotherapy followed by autologous stem cell transplant*). 5. Confirmed availability of tumor tissue (freshly collected biopsy preferred, archival tissue acceptable). 6. At least one bi-dimensionally measurable (≥ 1.5 cm) nodal lesion, or one bi-dimensionally measurable (≥ 1 cm) extranodal lesion, as measured on computed tomography (CT) scan. 7. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0, 1, or 2. 8. Adequate hematologic function and adequate renal function, defined as an estimated creatinine clearance ≥ 30 mL/min. 9. Negative SARS-CoV-2 antigen or PCR test within 7 days prior to enrollment.

4.2 Exclusion Criteria 1. Patient has failed only one prior line of therapy and is a candidate for stem cell transplantation; prior solid organ transplantation; or prior allogeneic stem cell transplant. 2. History of transformation of indolent disease to DLBCL; High-grade B-cell lymphoma with MYC and BCL2 and/or BCL6 rearrangements, and high-grade B-cell lymphoma not otherwise specified; or Primary mediastinal B-cell lymphoma. 3. History of severe allergic or anaphylactic reactions to humanized or murine monoclonal antibodies; or contraindication to obinutuzumab, rituximab, gemcitabine, oxaliplatin, or tocilizumab. 4. Prior treatment with glofitamab or other bispecific antibodies targeting both CD20 and CD3. 5. Peripheral neuropathy assessed to be Grade >1 (NCI CTCAE v5.0) at enrollment. 6. Treatment with radiotherapy, chemotherapy, immunotherapy, immunosuppressive therapy, or any investigational agent within 2 weeks prior to first study treatment; or treatment with monoclonal antibodies within 4 weeks. Corticosteroid therapy within 2 weeks prior to first dose of study treatment. 7. Primary or secondary central nervous system (CNS) lymphoma at recruitment or history of CNS lymphoma; current or history of CNS disease (e.g., stroke, epilepsy, CNS vasculitis, neurodegenerative disease). 8. Known active bacterial, viral, fungal, mycobacterial, parasitic, or other infection within 4 weeks; positive SARS-CoV-2 infection within 30 days prior, or documented within 6 months. 9. Positive

for hepatitis B virus (HBV), hepatitis C virus (HCV), or human immunodeficiency virus (HIV); suspected/latent tuberculosis; known/suspected chronic active Epstein-Barr viral infection, hemophagocytic lymphohistiocytosis (HLH), or progressive multifocal leukoencephalopathy. 10. Administration of a live, attenuated vaccine within 4 weeks before first study treatment. 11. Active autoimmune disease requiring treatment. 12. Recent major surgery (within 4 weeks) or clinically significant history of cirrhotic liver disease. 13. Adverse events from prior anti-cancer therapy not resolved to Grade 1 or better (excluding alopecia and anorexia).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Glofit-Gem0x arm receives a single dose of obinutuzumab 7 days prior to Cycle 1. This is followed by glofitamab step-up dosing (up to 30 mg) + Gem0x (gemcitabine [Trade names: Gemzar, Infugem; ATC Code: L01BC05] 1000 mg/m², oxaliplatin 100 mg/m²) for 8 cycles. This is followed by up to 4 cycles of glofitamab monotherapy. **Route of Administration:** Intravenous (IV). **Duration of Treatment:** 21-day cycles, for a maximum of 12 cycles (total 12 doses of glofitamab).

5.2 Concomitant & Prohibited Therapy Permitted: Tocilizumab IV as needed for the treatment of cytokine-release syndrome (CRS). **Prohibited:** Corticosteroid therapy within 2 weeks prior to the first dose of study treatment; administration of a live, attenuated vaccine within 4 weeks before the first study treatment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Tumor Assessments
Baseline	Day 0	Randomization, First Dose (Obinutuzumab pretreatment for experimental arm)
Interim	Every 21 Days (Cycles 1-12)	IV Administration, CRS Monitoring (overnight hospital stay on the first dose), Safety Labs, Efficacy Assessments
End of Study	6 Weeks Post-Treatment	Final Follow-up Visit, Exit Review, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Overall Survival (OS), defined as the time from randomization to date of death from any cause. **Measurement Method:** Clinical assessment evaluated via an Independent Review Committee (IRC).

7.2 Secondary & Exploratory Endpoints * **Progression-Free Survival (PFS)**: Defined as the time from randomization to the first occurrence of disease progression or death from any cause, as determined by the Independent Review Committee (IRC) and by the investigator. * **Complete Response (CR) Rate**: Defined as the proportion of patients whose best overall response is a CR on positron emission tomography/computed tomography (PET/CT) during the study, as determined by the IRC. * **Objective Response Rate (ORR)** and **Duration of Response (DOR)**. * **Safety/Toxicity profiles** (specifically incidence of CRS).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Assessed via regular investigator evaluations every 1 to 3 weeks during treatment cycles, specifically tracking CRS incidence and severity. **Serious Adverse Events (SAEs)**: Standard safety reporting timelines. **Data Monitoring Committee (DMC)**: Independent Review Committee (IRC) masked to treatment assignment to evaluate all response-based endpoints.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intention-to-Treat (ITT) for primary efficacy analyses; Safety Set for all patients receiving any study treatment. **Sample Size Justification**: \$N=270\$ randomized 2:1 to detect a statistically significant survival difference. **Handling of Missing Data**: Right-censoring for time-to-event outcomes (e.g., OS, PFS).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan**: Regular on-site and remote monitoring per sponsor standard operating procedures. **Audit Trail**: Independent Review Committee (IRC) evaluation and fully validated electronic data capture (EDC) systems.

11. Study Identifiers & Governance

Primary ID: {{G041944}} **Registry Number**: {{NCT04408638}} **Posting ID**: {{387901819878840696}} **Sponsor/Lead Organization**: {{Roche / Hoffmann-La Roche}} **Study Title**: {{STARGLO}} **Trial Status**: {{ACTIVE_NOT_RECRUITING (Active but not currently recruiting)}}

12. Clinical Framework (DLBCL Specific)

Primary Condition / Disease Area: {{Diffuse Large B-cell Lymphoma (Preferred UMLS Name: Diffuse large B-cell lymphoma)}} **Diagnosis Threshold:** {{Histologically confirmed R/R DLBCL after \$\ge 1\$ prior systemic therapy}} **Disease Definition:** {{Malignant lymphoma composed of large B lymphoid cells whose nuclear size can exceed normal macrophage nuclei, or more than twice the size of a normal lymphocyte. The pattern is predominantly diffuse. Most of these lymphomas represent the malignant counterpart of B-lymphocytes at midstage in the process of differentiation.}} **Semantic Type:** {{Neoplastic Process}} **Clinical Codes:** {{MeSH Code: D016403 | SNOMED ID: 109969005}} **Medical Methodology:** {{CD20xCD3 bispecific antibody combined with GemOx chemotherapy (Gemcitabine / L01BC05)}} **Optimization Target:** {{Overall Survival (OS) extension and disease remission}}

13. Study Procedures & Methodology

Management Pathway: {{Standard clinical oncology pathway for R/R DLBCL}} **Education Protocol (HCP):** {{Site initiation, CRS management/tocilizumab administration training}} **Education Protocol (Patient):** {{Informed consent, toxicity monitoring awareness (CRS)}} **Quality Audit Mechanism:** {{Independent Review Committee (IRC) for masked efficacy endpoint analysis}}

14. Observation & Follow-Up Schedule

Total Duration: {{Follow-up until death, study withdrawal, or trial closure}} **Onsite Visit Cadence:** {{Every 1 to 3 weeks during active 21-day cycles}} **Remote Monitoring Frequency:** {{As applicable during survival follow-up phase}} **Checklist Review Interval:** {{Per 21-day cycle}}

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					